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1 Table of contents

1 Table of contents 2
2 Abbreviations 4
3 Purpose 5
4 Clinical Investigational Sites 5
5 Study Enrollment Plan 5
6 Site Visits 6
6.1 Site Selection 6
6.1.1 Selection process 6
6.2 Site Initiation Visit 6
6.3 Monitoring 9
6.3.1 Centralized Monitoring 9
6.3.2 Site Follow up 9
6.3.3 On-site Monitoring Visit 10
6.3.3.1 Monitoring Visit frequency 11
6.3.3.2 Source Data Verification 11
6.3.3.3 Site Monitoring Visit preparation 12
6.3.3.4 Site Monitoring Visit 13
6.3.3.5 Short interview with the PI 13
6.3.3.6 Writing of monitoring report 14
6.4 Remote Monitoring Visit 15
6.4.1 Remote Monitoring Visit preparation 15
6.4.2 During the Remote Monitoring Visit 16
6.4.3 After the Remote Monitoring Visit 16
6.5 Site Close-Out Visit / Early termination 16
7 Data Management 19
7.1 Data Management team 19
7.2 Queries handling 19
7.2.1 Manual queries handling 19
7.2.2 Electronic queries handling 19
8 Communication 19
8.1 Sponsor Reporting 19
8.2 Non-Compliance 20
9 Study Documentation / Procedures 20
9.1 Investigator Regulatory File (e-IRF) 20
9.2 Patient Informed Consent 20
9.3 Case Report Forms (CRF) completion 21
9.4 Medical Records/Source documents 21
9.5 Adverse Events and Device Deficiencies Reporting 21
9.6 Source documents to be collected 22
9.6.1 Clinical Events Committee (CEC) 22
9.6.2 Corelab 22

DocuSign Envelope ID: 7E0A45D0-857F-4B83-A5B0-BF36A6FC45AD

9.7 Clinical Investigation Plan Deviations **23**
9.8 Investigational Device Management / Accountability **23**
9.9 Maintenance of study documents by sites **23**
10 Appendices **24**

DocuSign Envelope ID: 7E0A45D0-857F-4B83-A5B0-BF36A6FC45AD

2 Abbreviations

AE	Adverse Event
CERC	Cardiovascular European Research Centre
CIP	Clinical Investigation Plan
CRA	Clinical Research Associate
CPL	Clinical Project Leader
(e) CRF	(electronic) Case Report Form
COV	Close-Out Visit
DD	Device Deficiency
DM	Data Manager
EDC	Electronic Data Capture
GCP	Good Clinical Practices
IEC	Independent Ethics Committee
IRB	Institutional Review Board
(e) IRF	(electronic) Investigator Regulatory File
MA(C)E	Major Adverse (Cardiac) Event
MV	Monitoring Visit
PI	Principal Investigator
PIC	Patient Informed Consent
POT	Proximal Optimization Technique
SAE	Serious Adverse Events
SDV	Source Data Verification
SIV	Site Initiation Visit
SOP	Standard Operating Procedures
(e) TMF	(electronic) Trial Master File

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3 Purpose

The CERC (Cardiovascular European Research Center) is supporting CERIC in implementing and monitoring the clinical investigation entitled: KISS – Keep bIfurcation Single stenting Simple, a randomized, prospective, post-market, multicenter study to evaluate the potential benefit of side branch ballooning, in the setting of single stenting with systematic proximal optimization technique (POT), in the treatment of bifurcation lesion.

The monitoring is the act of overseeing the progress of a clinical study, and ensuring that it is conducted, recorded and reported in accordance with the Clinical Investigation Plan (CIP), Standard Operating Procedures (SOPs), Good Clinical Practices (GCP) and the applicable regulatory requirement(s).

The overall project management and coordination is the responsibility of the Sponsor: CERIC, 3 rue des Granges, 1204 Genève, Switzerland.

This study-specific Clinical Monitoring Plan provides to all Clinical Research Associates (CRAs) involved in this study with guidance and planning for the preparation, conduct, reporting and follow-up of the monitoring activities in relation to the study. This plan is complementary to the requirements contained in the approved protocol, applicable regulations, guidelines and Standard Operating Procedures.

The monitoring plan will be reviewed periodically throughout the study and updated as needed. The monitoring plan is a version-controlled document, therefore any changes to the Monitoring Plan will generate a new version of the document that will be reviewed and approved by the CERC and the Sponsor.

4 Clinical Investigational Sites

As defined in the Clinical Investigation Plan, 20 investigational sites participate in the clinical investigation in 6 countries. Each site is expected to recruit between 1 to 30 patients.

The detailed list of participating sites is provided in Appendix1.

5 Study Enrollment Plan

As per the Clinical Investigation Plan, the Enrollment Plan is as follows:

Total Number of patients expected to be enrolled in the study:	**600**
Estimated Start Date of enrollment:	Q2, 2020
Estimated End Date of enrollment:	Q2, 2021
Estimated End Date of the study:	Q3, 2022

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The recruitment is competitive. Each site is expected to recruit 30 patients with a limit of 60 patients per site. It will be possible for the site to include more than 60 patients after receiving a written approval from the sponsor.

6 Site Visits

6.1 Site Selection

6.1.1 Selection process

CERC and study PI provided a list of pre-selected sites for sending site selection questionnaires issued by the CERC Clinical Project Leader (CPL) and validated by:

- the Medical Director
- the Sponsor
- the study PI

The Clinical Project Leader sent the approved questionnaires to the pre-selected sites via email containing an invitation letter, the site selection questionnaire (pdf printable document and weblink) and the study synopsis.

Completed questionnaires were reviewed for completeness and answers were summarized in an excel spreadsheet that was sent to the study PI for review and final choice of the sites. The Clinical Project Leader informed the site of its qualification by email.

During the selection process, the Clinical Project Leader reviewed the essential documents received from the sites for completeness and accuracy.

6.2 Site Initiation Visit

Site Initiation Visits (either physical or by phone) are conducted by the CERC CRA(s) as per CERC SOP: CT-SOP-013 (Site Initiation Visit) Version 11.0 dated October 16, 2017.

Upon notification that essential documents are in place (i.e. getting of local regulatory approvals, signed contract…) with the clinical site to enable site initiation, the CERC CRA will schedule and conduct a site initiation visit.

The visit will be confirmed in writing (by SIV confirmation letter CT-SOP-013j and email) to the Investigator in order to confirm the visit date, location and agenda, to make sure that all necessary arrangements will be made at the site to allow the CRA to do his/her work properly.

Prior to the Site Initiation Visit, the Clinical Project Leader issues the IRF check-list listing all documents that should be available on site and transmits it to the CRA for preparing the visit.

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Then, the Clinical Trial Assistant, in collaboration with the CRA, prepares the Investigator's Regulatory File.

During the initiation visit the Principal Investigator and all study staff should be present. If for any reason, the Principal Investigator (PI) cannot be present, at least one Co-investigator must attend the meeting and a call will be scheduled with the PI to train him/her on the study. Inclusions in the site can start only when the PI has received an appropriate training and when the "Site regulatory greenlight" form (CT-SOP-013n) was appropriately filled in and validated by the Clinical Project Leader.

In the case a physical site initiation visit is not conceivable (e.g. site is inaccessible to CRA), a site initiation phone call can be planned after approval by the Sponsor.

During the Site Initiation Visit, the CRA:

- reviews and explains the following items to the site staff:
  - the Clinical Investigation Plan and all other study related procedures,
  - the study timelines and expectation on patient recruitment,
  - the process for collecting the patients' informed consent,
  - the process for collecting and reporting adverse events as defined in the Clinical Investigation Plan, as well as source documents to be sent to the CERC,
  - the investigator’s responsibilities,
  - the medical device presentation,
  - the completion of the study logs: Screening log form (CT-SOP-013g) and Patient Identification and Enrolment Log (CT-SOP-13d)
- trains the Principal Investigator and study staff on:
  - the study Case Report Form (Electronic Data Capture, EDC),
  - randomization procedure
  - Central / Core Lab procedure
- documents the training of the site personel on the registry by signing and requesting signatures on the Site Staff Training Form (CT-SOP-013m)
- collects / asks for the following documentation, if not already collected previously:
  - CVs of all site study team members,
  - the signed financial contract,
  - the original copy of the Clinical Investigation Plan signature page signed by the PI (2 copies should be signed, 1 original copy to be filed in the IRF and 1 for the TMF)
  - the original copy of the Instruction for Use signed by the PI (2 copies should be signed, 1 original copy to be filed in the IRF and 1 for the TMF)
  - the original copy of the e-CRF site access request form (CT-SOP-013i) dated and signed by each study team member who wishes to receive access to the eCRF,
  - the original copy of the site staff training form,
  - a copy of the site visit log (CT-SOP-014a) dated and signed by the PI,
  - a copy of the Delegation of Responsibilities log (CT-SOP-013e) dated and signed by all participating members,
  - a copy of the site contact list,

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  - completed Source Data Location List (CT-SOP-013o), dated and signed by the PI
- reviews source documents requirements and retention [15 years after final report and according to the local law]
- informs the investigator that an audit may be carried out during and/or after the end of the study and review his/her responsibilities to allow the representatives of the Regulatory Authorities and auditors mandated by the Sponsor to have a direct access to study documents, study data and source documents.
- the following items are not applicable for an initiation by phone and therefore will have to be done during the first visit on site:
  - checks the receipt of all relevant study supplies (Investigator Regulatory File, PICs)
  - reconciles and shows the Investigator Regulatory File to the site staff

The CRA will inform the CPL of any concerns or issues which will be escalated to the Sponsor as necessary. The CPL will assist the CRA in following-up the issue (action plan) until resolution and will inform the Sponsor about the outcome.

Once the site staff has been trained on the study and the documentation is in place, the CRA asks the Data Manager to create and send the eCRF login and password to each individual identified by the PI as requiring an access to the eCRF using the “EDC System Grant and Remove Access Request” form (DM-SOP-009a).

In cases where the EDC training could not be performed during the SIV, CRA or CERC CPL will train the site staff on the study and on the EDC after the SIV via a telephone conference/video.

After the Site Initiation Visit, the CRA:

- completes the “Site regulatory greenlight” form (CT-SOP-013n) to document that the site fulfills all requirements for study start and patient recruitment and, once the form has been signed off by CRA and CPL, the CRA informs the site accordingly
- completes the “Site Initiation Visit Report” (CT-SOP-013c) and the Issues Log (CT-SOP-014i) if any problems were identified during the visit, updates the IRF Check List as needed,
- issues the SIV follow-up letter (CT-SOP-013k) summarizing the most important items discussed during the SIV (enrollment plan, reminder to stay compliant to the protocol, safety requirements, ISO 14 155 and country specific regulations, reminder to complete the EDC in a timely manner, outstanding items identified during the visit with resolution requests…)

These documents will be sent to the CPL within 5 days after the date of the visit for review and approval.

Once the report is validated by the CPL (10 days after the date of the visit), the CRA sends the follow up letter to the site along with the Issues Log (CT-SOP-014i) for PI signature, within 15 working days after the date of the visit.

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Any issue must be resolved within 90 calendar days at the latest. Beyond this timeframe, the CRA requests the support of the CPL for resolving the issue(s).

The final version of the visit report along with its attachments will be filed in the eTMF.

A copy of the validated report along with its attachments will be kept in the e-TMF.

6.3 Monitoring

Monitoring is a quality control tool for determining whether study activities are being carried out as planned, so that deficiencies can be identified and corrected. Monitoring activities include communication with the Principal Investigator and study site staff; review of the study site's processes, procedures, and records; and verification of the accuracy of data submitted to the sponsor.

CRAs have a major responsibility in this process. Study data verification is the CRA's main task and accounts for most of the monitoring activities.

6.3.1 Centralized Monitoring

Centralized monitoring is a remote evaluation of the study data, at a location other than the sites at which the clinical investigation is being conducted. By reviewing important investigation activities, in real-time across sites, centralized monitoring seeks to identify higher risk clinical sites, with higher frequency of errors, protocol deviations or data anomalies, as well as to appraise site performance metrics.

For the purpose of KISS centralized monitoring, a risk assessment has been performed to identify critical data and processes that may jeopardize patient protection and data integrity.

Identified risks as well as how they will be monitored, managed, and mitigated are presented in Appendix 26.

Electronic critical data outlined in Appendix 26 are routinely reviewed to identify missing data, inconsistent data, data outliers, and potential protocol deviations (manual checks). This review is also populated in a Data Management report. The manual checks will be sent to CRAs once per month and will be used as a support for site follow up.

6.3.2 Site Follow up

The CRA should maintain a close follow-up with the sites to ensure adequate conduct of research and to maintain an efficient link with site study staff and facilitate the monitoring visit to come.

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To this end, each site should be contacted, at least once per month by phone and/or by e-mail by the CRA. A checklist with the items to be checked by CRAs during the follow-up will be provided by CPL.

Moreover, each site should also be contacted by the CRA after the first two enrolled patients to ensure protocol compliance.

Site follow up consists of the following:

- Motivate the site on enrollment and on the timely completion of the EDC / reporting of the events,
- Request missing essential study documents such as but not limited to, EC submission/approval/communication, any training record completed, updated CVs, copy of Delegation of Responsibilities/Training Log in case there is an updated in the study staff,
- Request missing source documents for endpoint related events and review them,
- Communicate study timelines,
- Communicate issues/pending actions highlighted by the centralized monitoring and provide support in their resolution,
- Perform additional training if deem necessary according to centralized monitoring results.

Depending on site performance metrics, site may be contacted more than once per month.

Each site follow up, even if performed by phone, shall be documented in an e-mail sent to the site which includes a summary of the data or activities reviewed; a description of any noncompliance, potential noncompliance, data irregularities, or other deficiencies identified; and a description of any actions taken, to be taken, or recommended. Such documentation should include the results of centralized monitoring activities.

All evidence for site FU and any relevant e-mail communication should be kept in the eTMF.

In case of detection of any issues or resolution of issues, the CRA should update the issues log, ensuring that the date of the issues log is made current. Any issue must be resolved within 90 calendar days at the latest. Beyond this timeframe, the CRA requests the support of the CPL for resolving the issue(s).

6.3.3 On-site Monitoring Visit

On-site monitoring is an in-person evaluation carried out by the CRA at the sites at which the clinical investigation is being conducted. On-site monitoring can identify data entry errors (e.g., discrepancies between source records and case report forms (CRFs)) and missing data in source records or CRFs; provide assurance that study documentation exists; assess the familiarity of the site's study staff with the protocol and required procedures; and assess compliance with the protocol and investigational product Site Monitoring Visits are conducted by the CERC CRA(s) as per CERC SOP: CT-SOP-014 (Monitoring visit) Version 10.0 dated October 16, 2017.

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6.3.3.1 Monitoring Visit frequency

It is expected that a total of three 1-day MVs per site should be performed at each site that included patients in the study.

Monitoring visits should be conducted according to this schedule:

- The first monitoring visit should be performed once 15 patients have been included at site (if applicable)
- The second monitoring visit should be performed once enrolment is closed at the site
- The third monitoring visit should be performed once all patients at this site have completed the 12-month FU.

If, for any reason, it appears that Monitoring Visits have to be performed more frequently (in one or several site(s)), the CERC CPL shall discuss it with the Sponsor. The Monitoring Plan and study budget shall be updated accordingly and agreed on in writing, following the same review and approval process as for the initial version.

Any additional Site Visit (e.g. a “for cause” visit) should be approved in writing on a case by case basis by the Sponsor.

The repartition of the monitoring visit might be changed in case of a high recruiting site or low recruiting site. Therefore, some sites might be monitored more than 3 times and others less than 3 times.

For sites with few data to be source data verified and with no outstanding issues highlighted in centralized monitoring, the third monitoring visit (to be performed once all patients at this site have completed the 12-month FU) may be done remotely, provided that all inform consents were reviewed during a prior monitoring visit. If required, source documents will be sent to the CRA by the center. This visit may also be fused with the COV of the site.

6.3.3.2 Source Data Verification

The correct completion of the PIC will be verified for all the included patients.

Source Data Verification (SDV) will be performed as per the Clinical Investigation Plan, applicable SOPs and study contract as described below:

- 100% of data will be source data verified in 10% of patients at each site (with a minimum of 2 patients per site)
- 100% of endpoints-related events will be source data verified (if not done remotely)

The 10% CRFs (patients) to be 100% monitored at each site will be randomly determined by using an excel file prepared by the CERC Data Manager. For this purpose, the CRA will perform the randomization that will be approved by the CPL before the visit.

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6.3.3.3 Site Monitoring Visit preparation

The CRA shall arrange an appointment with the PI and/or study staff and should attempt to schedule visits as far in advance as possible to allow for appropriate staff and records to be available.

The CRA will review the previous visit reports and follow up reports for any outstanding actions. If possible, these should be resolved prior to the visit as part of site follow up. In case it is not possible to resolve an outstanding action prior to the monitoring visit, the issue should be listed in the Interim Monitoring Visit Confirmation Letter.

The monitoring visit will be scheduled with the site and will be confirmed in writing (by letter or e-mail) to the Principal Investigator (PI) and eventually to the study staff (study coordinator, co-investigator, nurse etc.) in order:

- to confirm the visit date, location and agenda (including the timing of the interview with the investigator),
- to make sure that all necessary arrangements will be made at the site to allow the CRA to do his/her work properly including the direct access to the site records.

The monitoring visit confirmation letter (CT-SOP-014e) should include:

- the agenda of the visit,
- the list of patients who will be monitored during the visit and for which patients source documents should be available,
- the list of outstanding issues if applicable,
- the list of any other items that should be reviewed or collected (e.g. endpoints documentation, CVs…).

The CRA will inform the CPL as soon as the visit is scheduled.

List of items to prepare/to take for this visit:

- Clinical Investigation Plan and amendments (if applicable),
- Clinical Monitoring Plan,
- EDC guidelines,
- Initiation Visit Report or last Monitoring Visit report,
- IRF checklist,
- list of outstanding issues if applicable,
- list of outstanding queries if applicable,
- relevant correspondence since last visit,
- list of missing source documents for SAEs,
- site payment status
- Follow up reports

The CRA prepares a USB Key containing all necessary study documents to be provided to site, referring to the IRF Check-list (CT-SOP-014j).

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6.3.3.4 Site Monitoring Visit

Study data recorded in the EDC should be source data verified according to specifications provided in the Clinical Investigation Plan and in this Monitoring Plan.

In addition, data should be cross-checked for completeness, consistency, accuracy, and critically reviewed from a medical/clinical and scientific point of view.

During each monitoring visit, the CRA:

- ensures that the consent process is followed and documented in each subject's source notes and confirm that the correct version of the Ethics Committee approved Patient Informed Consent form was used and all required signatures and dates are obtained (100% monitoring) as required by ISO 14 155
- performs source data verification according to paragraph 6.3.3.2
- document issues, Clinical Investigation Plan deviations and corrective actions in the Monitoring Visit Report, in the "Issues log" and follow-up correspondence to the PI
- review and ensure that all endpoint related events, Clinical Investigation Plan deviations, and major compliance issues have been reported
- conduct site retraining as necessary
- verify that the site staff has submitted essential patient data and images to the corelab.
- review the "Screening log" (if applicable) and the "Patient Identification and Enrolment log"
- in case of any change in the site staff since last visit, verify that the "Delegation of responsibilities log", the "Site staff training form" and the "Site contact List" have been updated
- checks the study documentation available on site in the IRF and, when required collects:
  - updated dated and signed CV of the PI and all the site staff if needed,
  - a copy of the completed logs,
  - any missing document since last visit,
- signs and dates the Site visit log,
- alerts the CPL of any findings that are of concern.

6.3.3.5 Short interview with the PI

A short interview with the PI and his/her staff is highly recommended during the monitoring visit, in order to identify/discuss potential critical points noted by the CRA such as:

- ICF signature process
- EDC completion (any possible improvement measures)
- any change in study-related staff/organization or in delegation of investigator duties (reason, documentation),
- submission and on-site implementation of protocol amendments, if any,
- follow-up of corrective actions.

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and to inform the PI and his/her staff of study status and of any findings identified during the visit.

This discussion should be detailed in the Monitoring Visit Report.

If for any reason the PI is not available to attend this short interview, a telephone call will be scheduled with the PI to discuss above listed items. The report about this telephone conversation will be attached to the monitoring visit report. In the event the PI cannot be present for the two visits, the CRA should escalate to the CPL and the issue addressed with the PI.

6.3.3.6 Writing of monitoring report

Important findings should be immediately reported and no later than 1 working day after the monitoring visit by e-mail, to the CPL who will report them to the Sponsor, for example:

- Failure of the site to obtain written ICF
- Suspicion on data veracity or patients existence
- Unresolved / excessive outstanding queries
- Lack of understanding and/or non-compliance of study procedures by the site staff
- Major Clinical Investigation Plan deviations
- Or any issue judged by the CRA as an important finding

The CRA must document all important findings in the monitoring visit report.

After the monitoring visit, the CRA completes the “Monitoring visit report” (*CT-SOP-014b*) and issues a follow-up letter (*CT-SOP-014f*) to the site.

The monitoring visit report, its attachments (Issues Log and IRF Check List) and the follow-up letter will be provided to the CPL within 5 working days after the date of the visit for review and approval.

The Follow-up letters address at least the following items:

- Verification of PICs and issues noted, if applicable, along with recommended actions
- Monitored eCRFs identified by patient number
- Protocol/GCP-ISO 14155/country specific regulations deviations (if applicable) along with recommended actions
- IRF issues (if any)
- List of noted issues with recommended actions, assigned responsibilities and timelines for resolution

After the validation by the CPL, the CRA sends the FU letter, along with the completed "Issues log" for PI signature, to the site within 15 working days after the date of the visit.

Any issue must be resolved within 90 calendar days at the latest. Beyond this timeframe, the CRA requests the support of the CPL for resolving the issue(s).

DocuSign Envelope ID: 7E0A45D0-857F-4B83-A5B0-BF36A6FC45AD

The final version of the visit report along with its attachments will be filed in the eTMF.

A copy of the validated report along with its attachments will be kept in the e-TMF.

6.4 Remote Monitoring Visit

In the case an on-site monitoring visit is absolutely not conceivable (e.g. site is inaccessible to CRA), a remote monitoring visit should be planned after approval by the Sponsor. The remote monitoring consists of SDV or IRF consistency review performed remotely using documents sent by the site to the CRA.

6.4.1 Remote Monitoring Visit preparation

The visit should be organized as for an on-site visit (please refer to section 6.3.3. Site Monitoring Visit preparation) with the exception that site is informed that the visit is to be performed by web conference or via phone.

Prior the remote monitoring visit, the CRA calls the site to explain the organization of such particular visit, and that source documents will need to be pseudonymized, scanned and ready to be send to the CRA prior to the visit via CERC transfer platform). After the call, the detailed communication should be described in the adapted confirmation letter which includes the following information:

- the agenda of the visit,
- the list of patients who will be monitored remotely during the visit and for which specific patients source documents should be detailed to be uploaded,
- Details on how to transfer the source documents,
- Phone call access codes to join the remote monitoring visit,
- Remind the site that all endpoints events have to be declared prior the remote monitoring visit,
- the list of queries to be answered by the investigator if applicable,
- the list of any other items that should be reviewed or collected (e.g. endpoints documentation, CVs…).

For patients consents, site staff will be asked to show in front of the camera the consents. These will be monitored according to the following rules:

- For a first monitoring visit, all consents
- For a visit between 2 visits, all newly signed consents (never monitored) and non-conformed one previously verified
- For new versions of the consent (re-consenting), only the signature page of the consent will be monitored

In addition, the page of the medical file documenting the process of consenting and the date of first study-related procedure should be showed together with the consent.

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Regarding the patient medical files to send, only those containing data collected in the eCRF will be uploaded by site. The medical files should be pseudonymized by the site before sending them to the CRA via the CERC transfer platform. The patient files to monitor will respect the rules defined in section “6.3.3.3. Site Monitoring Visit preparation”.

6.4.2 During the Remote Monitoring Visit

Site and CRA will track for any eventual unreported or misreported event as per CIP requirements.

The monitoring visit should follow the rules defined in section “6.3.4. Site monitoring visit” whenever applicable.

During the remote monitoring visit, the site should remain available to help CRA resolve issues and answer queries.

6.4.3 After the Remote Monitoring Visit

The visit should be documented in a specific “Remote Monitoring Visit Report” (adapted from CT-SOP-014k). The rules for reporting however will be as described in section “6.3.6. Writing of monitoring report”. The same delays to draft and finalize the report remain applicable.

6.5 Site Close-Out Visit / Early termination

Site Close-Out Visit/ early termination are conducted by the CERC CRA(s) as per CERC SOP: CT-SOP-015 Close-Out visit (Version 9.0 dated October 16, 2017).

In general, a Site Close-Out Visit shall be conducted at each site after study completion (Last Patient Last Visit done) and the data cleaning process is completed (upon completion of source documentation verification for all eCRFs and after the site has resolved all final queries).

In the case a physical close-out visit is not conceivable (e.g. site is inaccessible to CRA), a site initiation phone call can be planned after approval by the Sponsor.

Prior to the visit, the Clinical Research Associate:

- checks that the appropriate study forms are available in the eTMF, including the Delegation of Responsibilities and Signature Log, the Site Visit Log, the training log form, the signature pages of all versions of the Clinical Investigation Plan;
- reviews the last Monitoring Visit Report and correspondence with the study site to identify any pending action items or outstanding documents to be collected,
- prepares the necessary documentation to update, if appropriate, the Investigator Regulatory File (to be sent by mail to the Investigator),
- verifies with the Clinical Project Leader if there are any specific points to be discussed with the Investigator (e.g. record retention, confidentiality, publication

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rights, financial agreement and payment of fees, device accountability and return of unused devices) during the phone call,

- contacts and arranges the phone call with the Investigator and any relevant site staff (e.g. Co-Investigator(s) and/or Study Coordinator and/or any site personnel involved in the clinical study (e.g. nurse(s)…)
- confirms the visit in writing to the Investigator and concerned staff, by sending the COV confirmation letter (CT-SOP-015b), which contains the agenda of the visit via e-mail.

The CRA should conduct the following activities at a minimum during the Site Closure or Early Termination Visit / call.

During the close-out visit, the CRA:

- addresses all open action items
- ensures that:
  - the Investigator is aware of his/her contractual obligations
  - all the invoices from the site have been collected and paid
  - the PI is aware of his continuing responsibility such as storage requirements, Sponsor's publication policy, confidentiality agreement and financial disclosure regulations
- reviews with the PI the status of ongoing AEs / SAEs,
- reviews completeness and accuracy of the documentation available on site regarding the IRF Checklist, for instance:
  - Delegation of responsibilities and signature log
  - Patient Identification and enrolment log
  - site staff training documentation
  - IRB/IEC correspondence to document approvals, SAEs, and major non-compliance reporting and annual updates
  - Monitoring Visit follow-up letters…
- collects the original copy of the completed logs (except for the Patient Identification and enrolment log) and make a copy of the logs to be kept in the IRF,
- if applicable, obtain or request a copy of the IRB/IEC close out letter to be sent to the sponsor
- dates and signs the Site Visit Log
- Review Informed Consents: location, execution, re-consenting if any and documentation

During the Close-Out Visit, the CRA discusses with the PI the status of any Serious Adverse Event (SAE) which was 'ongoing'/'unresolved' at the time of the last follow-up of the concerned patient.

If unforeseen circumstances prevent the PI from being present at the Study Closure or Early Termination visit / call, the CRA will detail the activities performed in a follow-up letter to the PI.

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The CRA will inform the CPL of any concerns or issues which will be escalated to the Sponsor as necessary. The CPL will assist the CRA in following-up the issue (action plan) until resolution and will inform the Sponsor about the outcome.

After the visit, the CRA issues the “Site Close-out Visit Report” (CT-SOP-015a), its attachments (Issues Log and IRF Check List) and the Close-Out Visit follow-up letter (CT-SOP-015c) to be provided to the CPL for review and approval within 5 days after the date of the visit.

The letters will address at least the following items:

- Summary of discussed items
- Issues and necessary actions identified during the visit
- List of collected documents
- Reminder of possibilities of audits or inspections after the close-out visit
- Information about procedures to inform the IRB/EC about the close-out
- Archiving requirements

Within 15 working days after the date of the visit, the CRA sends the follow up letter to the site, as well as the IRF Check-List and the completed "Issues log" (CT-SOP-014i) that should be dated and signed by the PI and filed in the IRF.

The final version of the visit report along with its attachments will be filed in the eTMF, which is accessible to the Sponsor throughout the course of the study.

A copy of the validated report along with its attachments will be kept in the e-TMF.

Once all outstanding issues/queries are resolved with the site, the CRA asks the Data Manager to remove the e-CRF access for all the site staff using the “EDC System Grant and Remove Access Request” form (DM-SOP-009a).

Upon finalization of the Clinical Study Report, the CPL sends a summary to the CRA to be forwarded to the sites before IRF archiving.

The CRA sends a final close-out letter to the site with the following documents in attachments:

- The Clinical Study Report summary;
- Data of patients enrolled at site (in electronic format);
- The final version of the Issues Log, if there were still pending issues at the time of the Close-Out which have been resolved**. All issues must be closed before archiving.**

The CRA requests to the site a copy of the close-out letter countersigned by the PI as acknowledgment receipt. All the documentation should be filed in the IRF before archiving.

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7 Data Management

7.1 Data Management team

CERC DM has been designated by CERIC for performing the Data Management activities in this clinical investigation.

7.2 Queries handling

The Clinical Research Associate, The Clinical Project Leader and the Data Manager are able to post queries on the EDC.

The CRA is responsible for following and closing his/her own queries as well as the ones issued by the data management according to the modalities described in the "Data Clarification Forms" SOP (DM-SOP-006).

7.2.1 Manual queries handling

As the data collection tool is an electronic CRF, this is not applicable.

7.2.2 Electronic queries handling

The CRA:

- generates queries in case of inconsistency with the source documents or with other data,
- informs the Investigators/study coordinator that there are pending queries and follows up the queries resolution using the corresponding reporting functionality of the e-CRF that tracks (by Patient, by Investigator or by Study Site):
  - edited queries (unresolved),
  - answered queries.

For each queries answered, the CRA:

- reviews the Investigator/study coordinator answer for consistency and as appropriate:
  - closes the query,
  - creates a new query if needed.

8 Communication

8.1 Sponsor Reporting

The sponsor will be made aware by the CPL of all major issues/deviations identified during the site visits.

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Original copies of reports are kept at CERC. and scans of these reports are filed in the eTMF which is accessible to the Sponsor throughout the course of the study. The eTMF will be sent to the Sponsor at the end of the study.

8.2 Non-Compliance

CRAs will notify the CPL as soon as possible (and no later than 1 working day) when significant non-compliance with the Clinical Investigational Plan or Good Clinical Practices is noted. The CPL will provide guidance to the CRA and include management staff as needed with the goal of clearly identifying problems and instituting corrective action. The CPL will provide same day communication with all relevant parties as necessary.

9 Study Documentation / Procedures

9.1 Investigator Regulatory File (e-IRF)

One Investigator Regulatory File is prepared per site under the CTA responsibility as per CERC SOP CT-SOP-013 Site Initiation Visit (Version 11.0 dated October 16, 2017).

An electronic copy of the Study Logs (e.g. Screening Log, Enrollment Log, Delegation of Responsibilities and Signature Log, etc.) prepared by the CTA and constituting each Investigator Regulatory File is kept at the CERC throughout the study duration.

An Investigator Regulatory File is provided to each site during the Site Initiation Visit and contains all the study documentation to be maintained up-to-date by the site under the Principal Investigator's responsibility.

The Investigator Regulatory File is reviewed by the CRA during on-site visits.

The detailed documents to be filed either in paper version or in an electronic version in the USB key is specified in the "IRF Check list" (CT-SOP-014j) and "Table of Contents Investigator Regulatory File" (CT-SOP-013b).

e-IRF will be proposed to sites and if the USB flash drive is not accepted by site, a paper IRF will be prepared.

9.2 Patient Informed Consent

Only the PI and Co-Investigators identified in the Delegation of Responsibilities and signature log are allowed and to obtain the written informed consent from the patient.

A copy of the signed Patient Informed Consent is filed in the corresponding patient's hospital chart. This is verified and documented by the CRA in the corresponding visit Reports.

No copy of the signed Patient Informed Consent will be made by the CERC CRA during their visits. Verification of the Patient Informed Consents at each visit is documented in the corresponding visit Reports.

Sponsor does not require that a copy of all the signed Patient Informed Consents be collected by the CRA during the Close-Out Visit after having been placed in sealed envelope(s), and properly identified, for archiving purposes at the Sponsor's offices.

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9.3 Case Report Forms (CRF) completion

Only the Investigators and designated authorized site staff are allowed to complete or modify data in the CRF (and Query Forms).

9.4 Medical Records/Source documents

The patient's medical records will be reviewed to ensure that the data collected in the Case Report Forms accurately reflects the patient's medical condition.

During the first on site visit, the Principal Investigator will be requested to fill in the "Source Data Location List" (CT-SOP-013o) specifying where medical record / source documents are located on site.

It is required from all the participating sites to have the patient's medical chart available for each Monitoring Visit even if the site is maintaining shadow charts, i.e. in the case a site uses computerized records, the site will be requested to print out the pertinent records (source data) and the hard copy shall be signed by the investigator. The hard copy is either filed with the CRF or (preferably) with the rest of the patient's medical chart.

If an electronic medical hospital file is used, all modification performed must be recorded by the system via an audit trail which must be available during the monitoring visit. The system validation certificate will have to be collected by the CRA ensuring that the audit trail is available and was appropriately tested.

If there is no audit trail:

- the investigator will be requested to print all the medical files of all participants, and will personally sign and date the first page of the print-out and indicate the total number of pages.
- during the on-site visits, the CRA will personally date and sign the first printout of the patient medical documents and indicate on the first page, the total number of pages.
- all print outs can then be stored with other study source documents or in the binder provided by the CERC.

9.5 Adverse Events and Device Deficiencies Reporting

Adverse events occurring within this clinical investigation are handled as per the Clinical Investigation Plan and the CERC SOP.

Device deficiencies (DDs) occurring within this clinical investigation are handled as per local regulatory requirements and local practices.

The following adverse events should be reported in the EDC:

- All endpoint-related adverse events (serious and not serious) will need to be reported and documented in the EDC. As much information as possible should be gathered with regard to the adverse event.
- All SAEs should also be reported and documented
- DDs will also be documented if they might have led to a SAE

Minor adverse events will not be recorded in the e-CRF.

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Safety reporting period is from the date of consent till the 12 months FU visit.

The PI (or designee) shall report all SAEs within the EDC system not later than 3 calendar days after knowledge of the event. The EDC will be programmed to send notifications to the CPL and other relevant personnel every time a new event is reported.

9.6 Source documents to be collected

9.6.1 Clinical Events Committee (CEC)

For CEC purpose, anonymized source documents have to be collected from the sites when one of the following endpoint-related event is declared:

- Death:
  - Death report and all available documents describing the cause of death. If available, event hospital discharge letter
  - Baseline discharge letter
- Myocardial Infarction:
  - ECG (pre & post index procedure, after MI occurrence/during hospitalization)
  - Cardiac enzymes (pre & post index procedure, after MI occurrence/during hospitalization)
  - Angiography films and reports (pre & post index procedure, after MI occurrence if available)
  - Hospital report/discharge letter for the baseline and at the time of the event
- Revascularization:
  - Angiography films and reports (pre & post index procedure, pre & post revascularization)
  - Surgery reports, if applicable
  - Hospital report/discharge letter
- Stent Thrombosis:
  - Same as for MI and Revascularization

and any other documents deemed necessary by the CEC members to be provided.

For cases, where no source documents are available, the site will be requested to write a narrative/Note To File describing what happened to the patient.

9.6.2 Corelab

CERC Core Laboratory has been designated by the Sponsor in order to centralize the analysis of angiographic images pre- and post-procedure (QCA analysis) and the analysis of angiographic images for stent thrombosis and revascularization events.

A specific procedure (Core Lab Plan) has been written by CERC to describe the detailed process for collecting, shipping and central analysis.

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The CRAs are in charge of following-up the retrieval of images and documents needed for Corelab analyses.

9.7 Clinical Investigation Plan Deviations

The following protocol deviations are identified for this study:

- Any deviation from subject inclusion and exclusion criteria or subject informed consent procedures.
- Patient Missed visit
- SAEs not reported by Investigators in the required timeframe as specified in the protocol
- Deviation from a protocol requirement such as follow-ups performed outside specified time windows, source documents permanently lost, etc.
- Cardiac enzyme pre- and/or post-procedure not done
- ECG pre- and/or post-procedure not done

The CRA will discuss and re-train the investigator and appropriate site staff on protocol procedures, deviations and document actions related to such deviations in the monitoring report.

Information on protocol deviations must be entered in the EDC system. All findings, issues and corrective actions will be followed by the CRA until resolution and documented in the "Issues log".

The Principal Investigator will be informed in writing of any Major Clinical Investigation Plan Deviations (Violations) noted during an on-site visit by a CRA in the visit follow-up letters. The PI will document his/her taken notice of this information by signing the “Issues log”.

Following a request to the DM, a deviation listing may be issued from the clinical database.

9.8 Investigational Device Management / Accountability

As contractually defined, the CERC is not in charge of the Investigational Device management and accountability.

9.9 Maintenance of study documents by sites

The CERC CRAs have to provide clear instructions to the sites during the close out visit with regard to maintenance of study records after completion of the study.

All study records have to be retained for 15 years after completion of the study, except if national applicable regulatory requirements are for a longer period.

In the event an investigator retires, relocates, or for any other reason withdraws from his/her responsibility of maintaining records for the period of time required, he/she will be reminded during the Site Close-Out Visit that custody of the records must be transferred to another person who will accept responsibility for the records, e.g. another physician or hospital member. Notice of such transfer must be given in writing to the Sponsor.

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10 Appendices

Appendix 1: List of participating sites
Appendix 2: CT-SOP-013 - Site Initiation Visit, Version 11.0 dated October 16, 2017
Appendix 3: CT-SOP-013j - SIV confirmation letter
Appendix 4: CT-SOP-013g - Screening log form
Appendix 5: CT-SOP-13d - Patient Identification and Enrolment Log
Appendix 6: CT-SOP-013m - Site Staff Training form
Appendix 7: CT-SOP-013i - e-CRF site access request form
Appendix 8: CT-SOP-014a - Site Visit Log
Appendix 9: CT-SOP-013e - Delegation of Responsibilities log
Appendix 10: Site Contact List
Appendix 11: CT-SOP-013o - Source Data Location List
Appendix 12: CT-SOP-013c - Site Initiation Visit Report
Appendix 13: CT-SOP-014i - Issues Log
Appendix 14: CT-SOP-013k - SIV follow-up letter
Appendix 15: CT-SOP-014 - Monitoring Visit, Version 10.0 dated October 16, 2017
Appendix 16: CT-SOP-014e - Monitoring Visit Confirmation Letter
Appendix 17: CT-SOP-014j - IRF checklist
Appendix 18: CT-SOP-014b - Monitoring Visit Report
Appendix 19: CT-SOP-014f - Monitoring Visit Follow-Up Letter
Appendix 20: adapted from CT-SOP-014k - Remote Monitoring Visit Report
Appendix 21: CT-SOP-015 - Close-Out visit, Version 9.0 dated October 16, 2017
Appendix 22: CT-SOP-015b - Close-Out Visit Confirmation Letter
Appendix 23: CT-SOP-015a - Site Close-Out Visit Report
Appendix 24: CT-SOP-015 - Close-Out Visit Follow-Up letter
Appendix 25: CT-SOP-013b - Table of Contents Investigator Regulatory File
Appendix 26: Manual checks and risks